



BOTSWANA NATIONAL HEALTH QUALITY STANDARDS FOR HOSPITALS

45. Psychiatric Volunteer Services

NAME OF HEALTHCARE FACILITY: _____

TYPE OF SURVEY: _____

DATE OF SURVEY: _____

DOCUMENT COMPLETED BY (Title and Name)

INTERVIEWEE (Name and Designation)

This Standard Assessment Manual (SAM) provides a means to record the level of compliance achieved for each criterion (NA NC PC C), comments explaining why compliance has not been achieved and actions required to achieve compliance. Each criterion has a guideline statement which provides clarification of the requirements for compliance.

In summary, criteria are scored as follows:

- 1. Compliant (C)** means the condition required is substantially met. Evidence of compliance should be present in a tangible and/or observable form, e.g. documentation, physical items, etc.
- 2. Partially Compliant (PC)** means the condition required is not totally met, but there is progress towards compliance.
- 3. Non-compliant (NC)** means there is no observable progress or minimal progress towards complying with the requirement specified in the criterion.
- 4. Not applicable (NA)** means the criterion will not be scored because the service is either not provided, or not required at the particular level of care the criterion is designed to measure.

Please circle the compliance rating assigned to each criterion. Explanatory comments should be provided for each criterion which does not receive a Compliant (C) score.

Documents Checked

Surveyor:

Surveyor:



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Overview of Psychiatric Volunteer Services

The health and social care system is under extreme pressure to improve the quality and efficiency of services. To meet the challenges ahead, service providers will need to think differently about how they work and who they work with.

One important group in these debates are the people who volunteer in health and social care, in both the voluntary sector and within public services. These people add significant value to the work of paid professionals and are a critical but often under-appreciated part of the health and social care workforce.

Research indicates that volunteers play an important role in improving people's experience of care, building stronger relationships between services and communities, supporting integrated care, improving public health and reducing health inequalities. The support that volunteers provide can be of particular value to those who rely most heavily on services, such as people with multiple long-term conditions or mental health problems.

The nature of volunteering is changing radically. Voluntary sector organisations are providing a growing range of public services, bringing volunteers into new aspects of service delivery. In some organisations, volunteers are increasingly being seen as an integral part of the care team.

The utilisation of volunteers provides a cost-effective means of supporting personnel as well as assisting in the enhancement of service delivery. Personnel at all levels should have the opportunity to be involved in planning effective utilisation of volunteers. A well-managed volunteer program can contribute to improved quality and quantity of services.



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Standards

45.1 Management and staffing

45.1.1 Standard

The volunteer service is organised, managed and staffed to provide a safe and effective service and is coordinated with other personnel and services within the organisation.

Intent of 45.1.1

The coordination of patient care depends on the exchange of information between the multidisciplinary team. This can be through verbal, written or electronic means according to appropriate policies determined by the organisation. Clinical leaders should use techniques to better integrate and coordinate care for their patients (for example, team-delivered care, multi-departmental patient care rounds, combined care planning forums, integrated patient records, case managers). The process for working together will be simple and informal when the patient's needs are not complex.

The patient, family and others are included in the decision process when appropriate.

The patient's record contains a history of all care provided by the multidisciplinary team and is made available to all relevant caregivers who are authorised to have access to its content.

45.1.1 Criteria

45.1.1.1	A suitably qualified and experienced health professional is identified with clearly defined and documented responsibilities for coordination of the volunteer service.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: The job description and qualifications will be reviewed during the personnel record audit.		

45.1.1.2	The manager is represented on the volunteers' management committee to act as liaison between the organisation and the volunteer service.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence is required.		

45.1.1.3	Recorded meetings between the organisation management and volunteers' management are held at least monthly.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence is required.		

45.1.1.4	There is a current organisational chart, which clearly states the agreed upon lines of communication, authority, responsibility and accountability.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence is required.		

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45.1.1.5	There is evidence that volunteers are selected in accordance with the necessary and appropriate skills to support the objectives of the volunteer service.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Evidence can be provided by copies of application forms, minutes of interviews, references etc.		

45.1.1.6	Volunteers are clearly identified by name badges.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Compliance will be measured by observation.		

45.1.1.7	Personnel employed in the volunteer service are assigned based on qualification, experience and interest.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Compliance will be measured during the volunteer file audit. A volunteer allocation roster should also be maintained.		

45.1.1.8	There is a mechanism to ensure that all volunteers work under the guidance of suitably qualified health professionals in the employ of the organisation.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Volunteers are not formal employees of the organisation and should therefore not be allowed to work unsupervised in order to safeguard patient safety.		

45.1.1.9	The volunteer service is supported by administrative, auxiliary and secretarial personnel who may themselves be volunteers.	Comment:
Severity: Moderate		
Category: Efficiency		
NA NC PC C		
Guideline statement: Evidence of administrative assistance to the volunteer service must be provided.		

45.1.1.10	There is a system in place to evaluate the performance of volunteers.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Volunteers are responsible for providing services on behalf of the organisation and the effectiveness of these services should be evaluated.		



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45.1.1.11	There is a system in place to provided acknowledgement and appreciation for services rendered by each volunteer.	Comment:
Severity: Moderate		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence is required.		

45.1.2 Standard

Volunteers sign a memorandum of agreement to abide by the conditions of the organisation.

Intent of 45.1.2

The aim of the Memorandum of agreement is to:

- Represent the legal contract between the organisation and the volunteer.
- Build and maintain a mutual understanding of each other's work;
- Keep each other well informed;
- Define working practices in the spirit of collaboration and partnership
- Strengthen volunteer management

45.1.2 Criteria

45.1.2.1	The agreement includes indemnity and waiver for injuries sustained on the premises.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: A copy of the memorandum of agreement for each volunteer must be available in the volunteer's file.		

45.1.2.2 CRITICAL	The agreement includes the maintaining of confidentiality and privacy of information.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: A copy of the memorandum of agreement for each volunteer must be available in the volunteer's file		

45.1.2.3	The agreement includes those functions, which may be carried out by volunteers.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: A copy of the memorandum of agreement for each volunteer must be available in the volunteer's file.		



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45.1.2.4	The agreement includes access to premises and facilities by volunteers.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: A copy of the memorandum of agreement for each volunteer must be available in the volunteer's file.		

45.1.2.5	Agreements are regularly reviewed and amended when required according to organisational policy.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: A copy of the memorandum of agreement for each volunteer must be available in the volunteer's file.		

45.2 Volunteer development and education

45.2.1 Standard

Management of the facility, in cooperation with volunteers, ensures that there is a written, planned and organised development and education programme for all volunteers.

Intent of 45.2.1

It is the responsibility of the organisation to ensure that volunteers are able to carry out the role they have been recruited to do, whatever that may be. The steps from recruitment to volunteers contributing effectively can be summarised as:

- Induction
- Assessing volunteer training and development needs
- Meeting needs, filling gaps
- Supervision and support

45.2.1 Criteria

45.2.1.1	There is written, planned orientation and induction programme for all new volunteers to the service.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Orientation is important to ensure that volunteers are aware of information relevant to the performance of their function and that they do not endanger patients and their safety for example, by breaching infection control practices or by inadvertently creating a risk, for example leaving a door open in a secure area.		

45.2.1.2	The orientation and induction programme introduces new volunteers to the relevant aspects of the volunteer service.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence of the implementation of a standardised orientation programme that is adapted for the various categories of volunteers is required.		



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45.2.1.3	The orientation and induction programme explains the relationship and line of communication between the volunteer service and the organisation.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence of the implementation of a standardised orientation programme that is adapted for the various categories of volunteers is required.		

45.2.1.4	The orientation and induction programme prepares volunteers for their roles and responsibilities.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence of the implementation of a standardised orientation programme that is adapted for the various categories of volunteers is required.		

45.2.1.5	The orientation and induction programme introduces volunteers to the policies and procedures of the volunteer service and the organisation.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence of the implementation of a standardised orientation programme that is adapted for the various categories of volunteers is required.		

45.2.1.6	The orientation and induction programme ensures the new volunteers are fully informed of the conditions of confidentiality and conditions of providing information to the media.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence of the implementation of a standardised orientation programme that is adapted for the various categories of volunteers is required. Evidence of signed confidentiality agreements must be available in the volunteer's file.		

45.2.1.7	All volunteers are trained regarding their role during a fire and emergency evacuation.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: The volunteer's participation in the organisation's overall fire safety and emergency evacuation processes will be evaluated.		

45.2.1.8	All volunteers are trained regarding their role in providing a safe and secure patient care facility.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		



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Guideline statement: This is of particular relevance to the mental health care setting where all volunteers must be alert to the safety and security of vulnerable patients. Documented evidence of training required.

45.2.1.9	The orientation and induction programme is reviewed annually, signed and dated.	Comment:
Severity: Moderate		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence is required.		

45.2.2 Standard

The service facilitates the development of volunteers to assume increasing responsibility and leadership within the volunteer service.

Intent of 45.2.2

The organisation has a responsibility to ensure that volunteers are educated in matters which affect their functioning.

Education should be relevant to each volunteer as well as to the continuing advancement of the organisation in meeting patient needs and maintaining acceptable volunteer performance, teaching new skills and providing training on new equipment and procedures. Where relevant, documented evidence should be available to demonstrate that volunteers who have attended training have acquired the necessary competencies, for example, in the use of newly-acquired medical equipment.

The leaders of the organisation should support the commitment to ongoing in-service education for volunteers by making space, equipment and time available for education and training programmes. Education and training can take place in a centralised location or in several smaller learning and skills development locations throughout the organisation.

45.2.2 Criteria

45.2.2.1	The volunteer service provides a relevant, written in-service training programme for volunteers.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence of an in-service training programme for volunteers will be required for compliance.		

45.2.2.2	There is a system in place to ensure that all volunteers attend in-service training programmes, and that records are kept.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence of training will be required for compliance. This can be held in the volunteer's file, training file, electronically, etc.		



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45.2.2.3	The volunteer service ensures that the volunteers are kept up to date with relevant new information.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence of training will be required for compliance. This can be held in the volunteer's file, training file, electronically, etc.		

45.2.2.4	The volunteer service manager ensures that all volunteers are able to initiate basic life support as necessary.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence of training will be required for compliance. This can be held in the volunteer's file, training file, electronically, etc.		

45.2.2.5	The volunteer service manager ensures that all volunteers are familiar with the organisation's internal and external fire and emergency evacuation plans and attend rehearsals at least annually.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence of participation will be required for compliance. This can be held in the volunteer's file, training file, electronically, etc.		

45.2.2.6	The volunteer service manager ensures that all volunteers are trained in the relevant health and safety policies and infection control policies.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence of training will be required for compliance. This can be held in the volunteer's file, training file, electronically, etc.		



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45.3 Policies and procedures

45.3.1 Standard

There are written policies and procedures for the activities of the volunteer service.

Intent of 45.3.1

Organisational leaders must ensure that all policies and procedures which apply to volunteers are available, implemented and that the implementation is monitored.

Policies and procedures relate to as a minimum:

- The recruitment of volunteer personnel
- The selection of volunteer personnel
- The orientation, induction and education of volunteers
- The scope and functions of volunteers
- Confidentiality and taking of photographs
- The raising funds by volunteers
- The accounting and auditing of finances accrued and managed by volunteers (where applicable)
- Volunteers accompanying residents out of the organisation
- Accidents and incidents in the volunteer service
- Information to the media

45.3.1 Criteria

45.3.1.1	The volunteer service has written policies and procedures to guide volunteers in the activities and management of the volunteer service.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Policies and procedures for at least a) – j) in the standard intent above are available and implemented.		

45.3.1.2	Volunteers are involved in the formulation of policies and procedures as well as protocols for volunteer service.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Minutes of meetings with volunteers must be provided.		

45.3.1.3	All policies and procedures are reviewed at appropriate intervals, dated and signed.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: The general approach is not to penalise the criterion where there are only a few documents that do not comply. In other words, the general trend should be taken into consideration. It is expected that policies will be reviewed whenever indicated because of changes, but time frames for the routine review of policies should be defined in organisation policy. It is generally accepted that a period of two to three years between reviews should not be exceeded.		



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45.3.1.4	Policies and procedures are kept in a manual, which is indexed and easily accessible.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: This may be interpreted to mean more than one manual – paper-based or electronic. The effectiveness of the indexing system needs to be tested by random checks on cross-references between manuals and policy documents. Accessibility should be tested at departmental level, especially in the case of electronic systems.		

45.3.1.5	There is a mechanism to ensure that all policies and procedures are known and implemented by all volunteers working in the volunteer service.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Compliance can be demonstrated in various ways as organisations have different ways of performing these tasks. Examples of making policies known may include: • Volunteers indicate their acknowledgement by signing on the reverse of each document, on the index or on a separate sheet/form, thus undertaking to keep up to date with all relevant policies • Key policies and procedures are discussed at meetings • Memos are circulated to inform volunteers of new/changed policy matters, etc. The criterion is not prescriptive regarding the method to be used but the system in operation should be tested for effectiveness. Volunteers do not necessarily have to receive formal in-service training on policies, but this often happens for important/key aspects.		

45.4 Facilities and equipment

45.4.1 Standard

Facilities and equipment are adequate to ensure the provision of safe, efficient and effective operation of the volunteer service.

Intent of 45.4.1

In order to provide safe patient care, each service requires adequate resources. The physical facilities required include adequate office accommodation for volunteers and adequate storage space for equipment.

45.4.1 Criteria

45.4.1.1	The volunteer service management personnel are included in the management committee of the organisation to advise on planning for equipment acquisition, deployment, utilisation and maintenance for the volunteer service.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence such as minutes of meetings will be required.		

45.4.1.2	Telephones are available for volunteers and visitors.	Comment:
Severity: Moderate		
Category: Structure		
NA NC PC C		



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Guideline statement: Compliance will be verified by observation.

45.4.1.3	There is an alarm/alert system in place in case of medical emergencies, which is known to all members of the volunteer service.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: A mechanism should be in place to ensure that assistance can be summoned rapidly and effectively should the need arise. Various methods such as an alarm, intercom, alert button or functional telephone/pager system is acceptable. The mechanism to be used should be documented in a policy and new team members educated regarding the content of the policy.

45.4.1.4	There is an alarm system in place in case of security threats, which is known to all members of the volunteer service.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: Alarm systems can include intruder detection systems, security guards and panic alarms. Security guards must be monitored to ensure that they are providing an effective access control system. Alarm systems may be replaced by CCTV.

45.4.1.5	There is office facility available for the volunteer service manager.	Comment:
Severity: Moderate		
Category: Structure		
NA NC PC C		

Guideline statement: Compliance will be verified by observation.

45.4.1.6	There is adequate space for patient/group work by the volunteer service.	Comment:
Severity: Serious		
Category: Structure		
NA NC PC C		

Guideline statement: Compliance will be verified by observation.

45.4.1.7	There is adequate light and ventilation in therapy areas.	Comment:
Severity: Serious		
Category: Structure		
NA NC PC C		

Guideline statement: Compliance will be verified by observation.

45.4.1.8	There is adequate and suitable equipment to provide appropriate stimulation to the patients.	Comment:
Severity: Serious		
Category: Structure		
NA NC PC C		



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Guideline statement: Compliance will be verified by observation.

45.4.1.9	There is adequate storage space for all equipment, appliances and tools.	Comment:
Severity: Moderate		
Category: Structure		
NA NC PC C		

Guideline statement: Compliance will be verified by observation.

45.4.1.10	All furnishing and equipment are clean and in good condition to ensure the safety of patients and volunteers.	Comment:
Severity: Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: Compliance will be verified by observation.

45.5 Patient care

45.5.1 Standard

Within the existing constraints of the organisation, volunteers aim to promote the psycho-social, physical and spiritual wellbeing of the patients.

Intent of 45.5.1

Volunteer services must be managed to ensure safe and effective service delivery, coordinated with other services within the organisation.

45.5.1 Criteria

45.5.1.1	The functions of the volunteers relating to patient care are clearly defined and supervised.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Volunteers are not formal employees of the organisation and should therefore not be allowed to work unsupervised in order to safeguard patient safety.

45.5.1.2	The volunteer services bring specific matters relating to the health of patients to the attention of the medical and/or nursing personnel.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: Documented evidence is required.



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45.5.1.3	Reports on volunteer involvement in group and individual resident stimulation are provided by volunteers to support further clinical decisions.	Comment:
Severity: Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence is required.		

45.5.1.4	Reporting on patient progress to the multidisciplinary team is done on a regular basis.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Documented evidence is required.		

45.6 Quality improvement

45.6.1 Standard

A formalised pro-active quality improvement approach is maintained by volunteers.

Intent of 45.6.1

It is the responsibility of the management of the organisation to ensure that standards are set throughout the organisation. Within each department or service, it is the responsibility of managers to ensure that standards are set for the particular department. This requires coordination with the organisation's steering committee. Departmental managers use available data and information to identify priority areas for quality monitoring and improvement.

The following will be evaluated:

- The manner in which problems were identified and prioritised in this service for which quality improvement activities were initiated
- The processes put in place to resolve the problems
- The identification of indicators to measure improvement
- The tool(s) used to evaluate these indicators
- The monitoring of these indicators and corrective steps taken when goals were not achieved
- Graphed and/or tabled results, as appropriate

45.6.1 Criteria

45.6.1.1	There is a written quality improvement programme for the volunteer service that is developed and agreed upon by the personnel of the service.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: 45.1.1.1 There is a written quality improvement programme for the volunteer service that is developed and agreed upon by the personnel of the service. The minimum requirement for compliance will be the availability of evidence of: • At least one clinical audit (for volunteers this would require demonstrating that at least one of the quality improvement projects improved patient care) • Participation in documentation (patient record) audits • Participation in the monitoring of near misses, adverse events and sentinel events		



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45.6.1.2	Indicators of performance are identified to evaluate the quality of treatment and patient care.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: As the emphasis is on the quality of patient care, this cannot be compliant if there is no evidence of clinical audits. These could take the form of evaluation of the implementation of clinical guidelines, case discussions, reports on negative incidents, etc. An indicator, in this context, is a measure used to determine improvements in clinical care over time. The information gained must be used to identify and implement improvement processes. Statistical reports without documented evidence of continuous quality improvement will be scored PC.

45.6.1.3	Processes are selected in order of priority for evaluation and improvement in the quality of treatment and care.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Quality monitoring is typically applied to high risk, high volume or high cost activities, or areas of concern identified by personnel, patients or visitors.

45.6.1.4	The quality improvement cycle includes the monitoring and evaluation of the standards set, and remedial action implemented.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: The quality improvement cycle does not stop with the implementation of a quality improvement plan. Ongoing monitoring of the process or procedure under consideration is required, with evaluation of the monitoring data collected to ensure that the plan has achieved its intended aim. If not, the quality improvement cycle should be repeated, incorporating the learning from previous attempts, until the desired aim is achieved.

45.6.1.5	A documentation audit system is in place.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Documented evidence of such audits must be provided at departmental level. Where the documentation (patient record) audit is an organisation-wide multidisciplinary process, which includes all clinical departments, this criterion will be scored according to the findings of the involvement of each department in the process. Evaluation of results and remedial action taken must be documented as well as maintenance of achieved improvements over time.

45.6.1.6	An incident managing system for monitoring near misses/adverse events/sentinel events is implemented, which includes the documentation of responses to recorded incidents and interventions to prevent recurrence of the incident or minimise harm in the event of a recurrence.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Participation at department level in the organisation's overall system of the monitoring of near misses/adverse events/sentinel events will be evaluated. Analysed data, with responses/remedial action related to departmental incidents are required.



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45.7 Patient rights

45.7.1 Standard

The organisation is responsible for providing processes that support patient and family rights during volunteer care.

Intent of 45.7.1

This refers to the implementation of organisational policy on patient and family rights. Compliance will be verified during observation of patient care processes, patient record audits and patient interviews.

45.7.1 Criteria

45.7.1.1	There are processes that support patient and family rights during volunteer care.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: The organisation's policies will be evaluated against country-specific legislation, where applicable. The implementation thereof will be evaluated during the auditing of patient records, patient interviews and observation during a walk-about in the department. This applies to all the relevant criteria below.		

45.7.1.2	There are processes to ensure that volunteer care is respectful of the patient's personal values and beliefs.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Implementation will be evaluated during the auditing of patient records, patient interviews and observation during a walk-about in the department.		

45.7.1.3	Measures are taken to protect the patient's privacy, person and possessions.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Confidentiality of patient information and patient privacy must be protected at all times. Standard operating procedures specific to the conditions and processes in the unit must be in place, known to relevant personnel and implemented. Likewise, security measures specific to the unit must be in place to safeguard the patients' person and possessions.		

45.7.1.4	Volunteers respect the rights of patients and families to treatment and to refuse treatment.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit and patient interviews.		



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45.8 Prevention and control of infection

45.8.1 Standard

There is a comprehensive infection prevention and control programme encompassing patient care and employee/volunteer health.

Intent of 45.8.1

This standard measures the implementation of the organisational policy for infection prevention and control at unit level as well as the identification of unit-specific infection control risks. Volunteers must be aware of the policy and there must be evidence that it is implemented throughout the organisation.

45.8.1 Criteria

45.8.1.1	The organisation identifies the procedures and processes associated with the risk of infection during volunteer services and implements strategies to reduce risk.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: The strategies to reduce risk should be defined in policies and procedures and include the risks listed in this standard as a minimum. Such documentation must form part of the organisation-wide infection control processes.		

45.8.1.2	The organisation provides education on infection control practices to volunteers.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence of personnel training will be required. Information pamphlets, posters, individualised education etc. will provide evidence of volunteer education		

45.8.1.3	Volunteers are trained in correct hand washing procedures.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence of volunteer training will be required, together with evidence of monitoring of implementation, for example, hand washing audits performed regularly according to organisational policy.		

45.8.1.4	Organisational policy on handling, storage and disposal of healthcare waste is implemented.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		



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45. Psychiatric Volunteer Services

Guideline statement: The implementation of the organisation's master policy will be evaluated in the department. The processes must include handling at source, collection, storage and disposal of healthcare waste.